

This factsheet is about having a colonic stent fitted. It explains why you might have a stent fitted, what's involved and the possible complications.

What is a colonic stent?

A colonic stent is a small flexible tube that is inserted into your bowel to keep the bowel open when it has become blocked (obstructed) or partly blocked by bowel cancer. The stent is made of a metal mesh, sometimes coated with silicone. A stent doesn't treat the cancer but it can help to manage the symptoms of the blockage.

Why are stents used?

If your bowel has become blocked, it may be difficult for your bowel movements to pass through. This can cause pain, cramp, bloating and/or sickness. A stent can relieve these symptoms by widening the bowel to open and unblock it. This allows your bowel to empty more easily again.

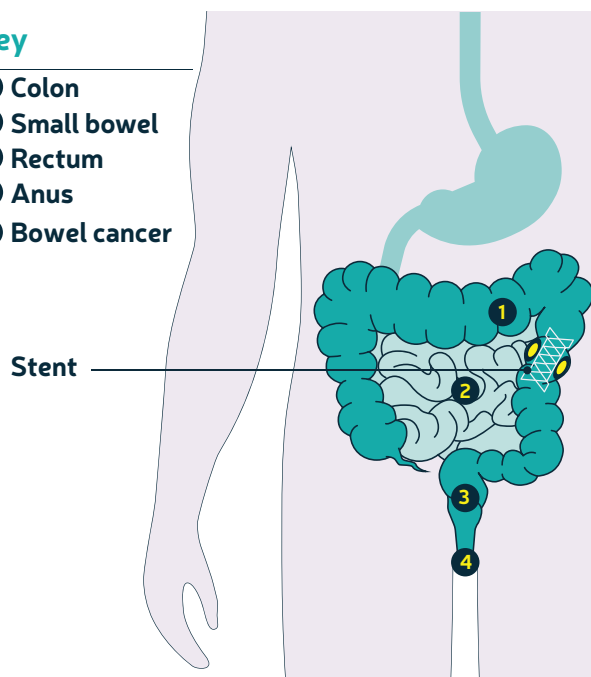
Stents can be temporary or permanent. You might be offered a temporary stent if you are going to have surgery later to remove the cancer that is blocking your bowel. By relieving your symptoms, it gives you and your doctor more time to prepare for surgery. Some research suggests this can mean less time in hospital or intensive care and lead to fewer complications. Having a stent fitted might also reduce the need for a stoma.

If surgery isn't suitable for you at this time or you choose not to have it, you might be offered a permanent stent to relieve your symptoms. You can discuss this with your specialist nurse.

Key

- ① Colon
- ② Small bowel
- ③ Rectum
- ④ Anus
- Bowel cancer

Stent



Before your stent is fitted

You will have a CT (computed tomography) scan to see exactly where the blockage is. You will be asked to provide consent to have your stent fitted. This is a good time to ask further questions or express any concerns you may have about the procedure. You may need to be monitored after the procedure. This may mean you need to stay in hospital for up to 48 hours.

When you get to the hospital, you will probably have an enema. This is where liquid is inserted into your bottom (anus) to clear the bowel below the blocked area. You might be given an injection of antibiotics through a needle (cannula) in your hand to help prevent infection before you have your stent fitted.

About colonic stenting

Fitting your stent

Your stent will be fitted in either the X-ray or endoscopy department. You'll be asked to lie on your side on a treatment table. You will be awake but you might be offered a sedative to help you stay relaxed. Let the doctor know if you feel pain or feel sick as they can give you medicines to help.

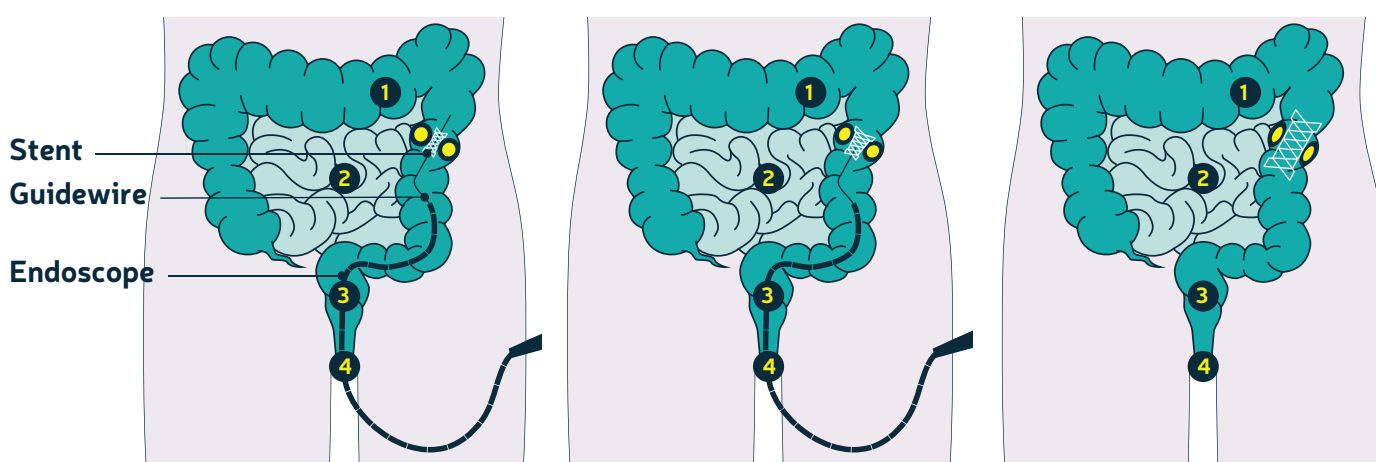
Your stent will be fitted by a radiologist and/or an endoscopist. A radiologist is a doctor who specialises in using scans such as X-rays. An endoscopist is a doctor or nurse who specialises in using endoscopes. Endoscopes are tubes with a camera and a light on the end and they're used to see inside the body.

The endoscopist will place an endoscope into your bottom and through your bowel until it reaches the blockage. They will pass the stent through the endoscope using a guidewire to help.

The endoscopist will use images on a screen (from the endoscope and X-rays) to see inside your bowel.

Fitting your stent usually takes between 45 and 90 minutes. It can sometimes take more than one procedure to get the stent in the right place.

Having your stent fitted



Step one

Your doctor will place the stent inside the blocked area using an endoscope and guidewire. The stent is very narrow to start with, about the thickness of a pen.

Step two

The stent will start to open up. After the stent has opened up the endoscope will be removed.

Step three

The stent will slowly expand until it is about 3cm wide to hold the bowel open. Your bowels will then be unblocked.

Key

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| 1 Colon | 4 Anus |
| 2 Small bowel | ● Bowel cancer |
| 3 Rectum | |

About colonic stenting

After your stent is fitted

After the procedure you will be closely monitored for the first four to six hours. You might have an X-ray 24 hours after your stent is fitted to see if it's in the correct position and has expanded properly. You may need to stay in hospital for up to 48 hours to make sure your bowel is working again. If you have a permanent stent fitted, you will have follow-up appointments over the following few weeks to make sure it is working properly.

Possible risks with having a stent

Most people don't have any serious problems caused by their stent. Your doctor or specialist nurse will explain the possible risks. They may include:

- **Stent failure** – sometimes the stent can't be fitted in the right place or doesn't open the bowel enough and the bowel doesn't start working again. If this happens, your doctor will need to stop the procedure and discuss other options with you
- **Perforation** – this is when fitting the stent tears the bowel wall. This is uncommon but can be a serious complication. It can mean you need an operation to remove the damaged part of the bowel
- **Migration** – this is where the stent comes loose from its position and moves out of place. If this happens your bowel could become blocked again. You might need to have the stent removed or replaced
- **Re-obstruction** – this is where the tumour grows through the mesh of the stent, blocking your bowel again. This might need treating with another procedure such as a new stent fitted
- **Bleeding from your bottom** – it's normal to notice a little bit of blood after the stent is fitted and it's usually nothing to worry about. If you notice a lot of bleeding, tell your specialist nurse straight away
- **Mild discomfort or pain** – some discomfort or pain is normal as your bowel starts working normally again. You may feel discomfort in your tummy (abdomen) or your rectum depending on where your stent is fitted

Contact your specialist nurse straight away if you have any of the following symptoms, as they could be a sign of a problem that needs treating quickly:

- Pain or tenderness of the tummy (abdomen) that is more than mild or uncomfortable
- A lot of bleeding from your bottom
- Fever, a faster heart rate, faster breathing or confusion
- You become constipated, feel sick, or your tummy becomes uncomfortable or bloated with spasms or gurgling

About colonic stenting

Will there be any changes to my bowel habits?

It is likely that your bowel habits will be different after you have a stent fitted. The changes vary from person to person, and range from small changes to bigger problems that could have more of an effect on your life.

You may experience:

- diarrhoea
- loose stools
- frequent small bowel movements
- less control of your bowel movements
- soreness
- a small amount of bleeding from your bottom

You might be given laxatives to keep your bowel moving and poos soft or slightly loose after your stent is fitted. These can lead to diarrhoea and wind. Your nurse will keep adjusting the dose to allow any problems to settle whilst keeping your bowel moving.

Get support

Contact your specialist nurse or GP if you're having problems that don't improve. There are medications to help with diarrhoea and exercises that can help to improve your bowel control.

Managing your diet

You may need to eat softer foods for a few days after your stent is fitted. Start with just liquids, then introduce some soft foods such as porridge, smoothies, well-cooked casseroles and soups. You may find it helpful to avoid eating anything hard like raw vegetables, nuts and seeds.

Your specialist nurse will explain more before you go home. You can read more about diet and bowel cancer at bowelcanceruk.org.uk

Are there other treatment options?

Depending on the locations of the tumour a stent is not always possible. If your bowel is blocked, you might be offered surgery to remove the tumour or blocked section of bowel straight away without having a stent fitted.

Some people choose not to have treatment with a stent or surgery. If your bowel is blocked, it might be possible to manage your symptoms with laxatives, a low-fibre diet and pain relief. Your doctor might talk to you about this if the aim of your treatment is to provide comfort rather than to remove the cancer.

It is important that you understand and are happy with the treatment suggested for you and any possible side effects it may cause.